ONC-305-301

Clinical Study Analysis Report

Pharma-style simulated statistical report aligned to CSR sections

Document	Clinical Study Analysis Report	Study	ONC-305-301
Version	2.0	Status	Final simulated package
Data cut-off	2026-01-31	Database lock	2026-02-15
Author	Luke Johnston	Classification	Portfolio demonstration
Purpose	Biostatistics/statistical-programming portfolio	Data	100% simulated; no patient data

Important: this is a simulated portfolio package designed to demonstrate biostatistics, statistical programming, reproducibility, and regulatory-document writing. It is not a real clinical trial and is not clinical evidence.

Document administration

Version	Date	Description
1.0	2026-02-10	Initial portfolio statistical report
2.0	2026-02-15	CSR-style report with study conduct, efficacy, safety, QC and reviewer appendices

Report synopsis

Item	Result / description
Study	ONC-305-301; simulated Phase III oncology portfolio
Data cut-off / DBL	2026-01-31 / 2026-02-15
Subjects	420 randomized: 211 ONC-305 + SOC and 209 Placebo + SOC
Primary endpoint	PFS HR 0.69 (0.56, 0.86); log-rank p-value <0.001
Key secondary endpoints	OS HR 0.67 (0.51, 0.88); log-rank p-value 0.004; ORR 97 (46.0%); 95% CI 39.1-52.9% vs 41 (19.6%); 95% CI 14.5-25.7%
Safety	Any TEAE 203 (96.2%) vs 186 (89.0%); serious TEAE 65 (30.8%) vs 53 (25.4%); grade >=3 TEAE 86 (40.8%) vs 76 (36.4%)
QC status	62 PASS, 0 WARN, 0 FAIL

This report is written as a portfolio-scale statistical report, not a complete regulatory CSR. It includes the sections a recruiter would expect a statistician to understand: populations, endpoint definitions, statistical methods, efficacy, safety, data standards, QC and limitations.

1. Study design and analysis methods

• Randomized, double-blind, placebo-controlled Phase III oncology simulation.

• The primary endpoint is PFS analyzed in the FAS using KM summaries, log-rank test and Cox model.

• OS and ORR are key secondary endpoints; safety is summarized in the Safety Set using actual treatment received.

• All analysis datasets, TLF outputs and reports are generated reproducibly from synthetic data.

Endpoint	Population	Primary method	Result location
PFS	FAS	KM, log-rank, Cox HR	Table 14.2.1; Figure 14.2.1
OS	FAS	KM, log-rank, Cox HR	Table 14.2.2; Figure 14.2.2
ORR/DCR	FAS	Counts, percentages, exact CI	Table 14.2.3
Safety	Safety Set	Subject-level TEAE and lab summaries	Tables 14.3.x; Listings 16.2.x

2. Subject disposition and baseline characteristics

Category	ONC-305 + SOC (N=211)	Placebo + SOC (N=209)	Total (N=420)
Randomized	211 (100.0%)	209 (100.0%)	420 (100.0%)
Full analysis set	211 (100.0%)	209 (100.0%)	420 (100.0%)
Safety set	211 (100.0%)	209 (100.0%)	420 (100.0%)
Ongoing	12 (5.7%)	9 (4.3%)	21 (5.0%)
Completed treatment	10 (4.7%)	9 (4.3%)	19 (4.5%)
Progressive disease	156 (73.9%)	173 (82.8%)	329 (78.3%)
Adverse event	5 (2.4%)	1 (0.5%)	6 (1.4%)
Withdrawal by subject	6 (2.8%)	0 (0.0%)	6 (1.4%)
Death	14 (6.6%)	16 (7.7%)	30 (7.1%)
Other	8 (3.8%)	1 (0.5%)	9 (2.1%)

Baseline characteristics excerpt

Characteristic	ONC-305 + SOC (N=211)	Placebo + SOC (N=209)	Total (N=420)
Analysis set: Full analysis set	211	209	420
Age, years - mean (SD)	62.9 (9.1)	61.1 (9.4)	62.0 (9.2)
Age, years - median (min, max)	62.0 (40.0, 82.0)	61.0 (34.0, 82.0)	62.0 (34.0, 82.0)
Age group			
<65	128 (60.7%)	133 (63.6%)	261 (62.1%)
>=65	83 (39.3%)	76 (36.4%)	159 (37.9%)
Sex			
Male	115 (54.5%)	120 (57.4%)	235 (56.0%)
Female	96 (45.5%)	89 (42.6%)	185 (44.0%)
Race			
Asian	140 (66.4%)	108 (51.7%)	248 (59.0%)
White	53 (25.1%)	78 (37.3%)	131 (31.2%)
Black or African American	11 (5.2%)	11 (5.3%)	22 (5.2%)
Other	7 (3.3%)	12 (5.7%)	19 (4.5%)

3. Data review and protocol deviations

• A data-review log and protocol-deviation table are included to show realistic database-lock thinking.

• Deviation counts are mock/simulated and are not derived from real clinical data.

Major protocol deviation category	ONC-305 + SOC (N=211)	Placebo + SOC (N=209)	Total (N=420)
Any major protocol deviation	12 (5.7%)	16 (7.7%)	28 (6.7%)
Inclusion/exclusion criterion deviation	3 (1.4%)	5 (2.4%)	8 (1.9%)
Missed or out-of-window disease assessment	6 (2.8%)	8 (3.8%)	14 (3.3%)
Prohibited concomitant medication	3 (1.4%)	2 (1.0%)	5 (1.2%)
Study drug interruption > 28 consecutive days	2 (0.9%)	3 (1.4%)	5 (1.2%)
Potential unblinding event	0	0	0

Data review issue log

Issue ID	Area	Issue	Observation	Resolution	Status
BDR-001	Data review	Missing response assessments	29 subjects have NE BOR by simulation design	Handled as NE; ORR denominator remains all FAS subjects	Closed
BDR-002	Endpoint	PFS censoring traceability	Subjects without progression/death censored at last adequate assessment	CNSR documented in ADTTE and SAP	Closed
BDR-003	Safety	AE dates relative to exposure	All TEAEs checked for chronology against treatment start/end	Automated chronology checks passed	Closed
BDR-004	Programming	TLF reproducibility	All outputs regenerated from a single run_all.py command	Manifest and validation tracker generated	Closed
BDR-005	Metadata	Reviewer traceability	Source-to-ADaM traceability matrix added	ADaM reviewer guide created	Closed

4. Primary efficacy: progression-free survival

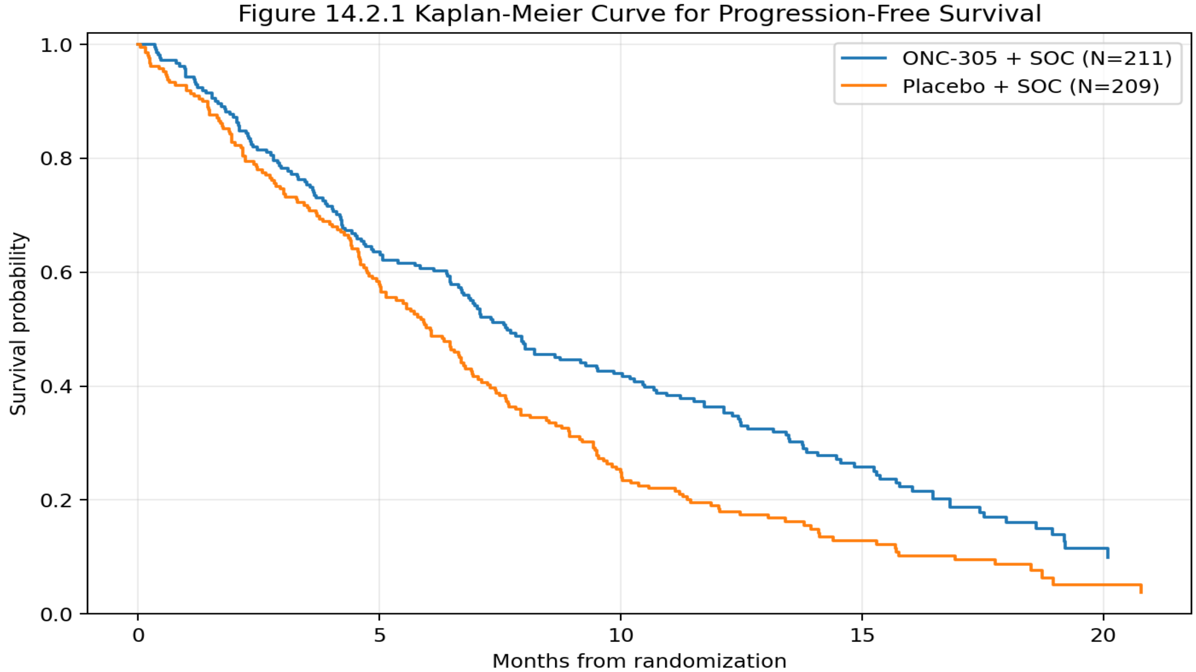
• The simulated primary analysis favors ONC-305 + SOC, with Cox HR 0.69 (0.56, 0.86) and log-rank p-value <0.001.

• Kaplan-Meier medians and landmark rates are reported by treatment arm.

• The subgroup forest plot is descriptive and intended to demonstrate oncology TLF competence.

Analysis	N	Events, n (%)	Median months (95% CI)	6-mo KM rate	12-mo KM rate	18-mo KM rate
ONC-305 + SOC	211	170 (80.6%)	7.7 (6.9, 9.5)	60.7%	36.3%	16.1%
Placebo + SOC	209	189 (90.4%)	6.1 (5.4, 6.6)	50.2%	19.0%	8.7%
Treatment comparison						
Cox HR, treatment vs control			0.69 (0.56, 0.86)			
Adjusted Cox HR			0.67 (0.54, 0.82)			
Log-rank test p-value			<0.001			

Figure 14.2.1 Kaplan-Meier Curve for Progression-Free Survival



5. Secondary efficacy: OS and response

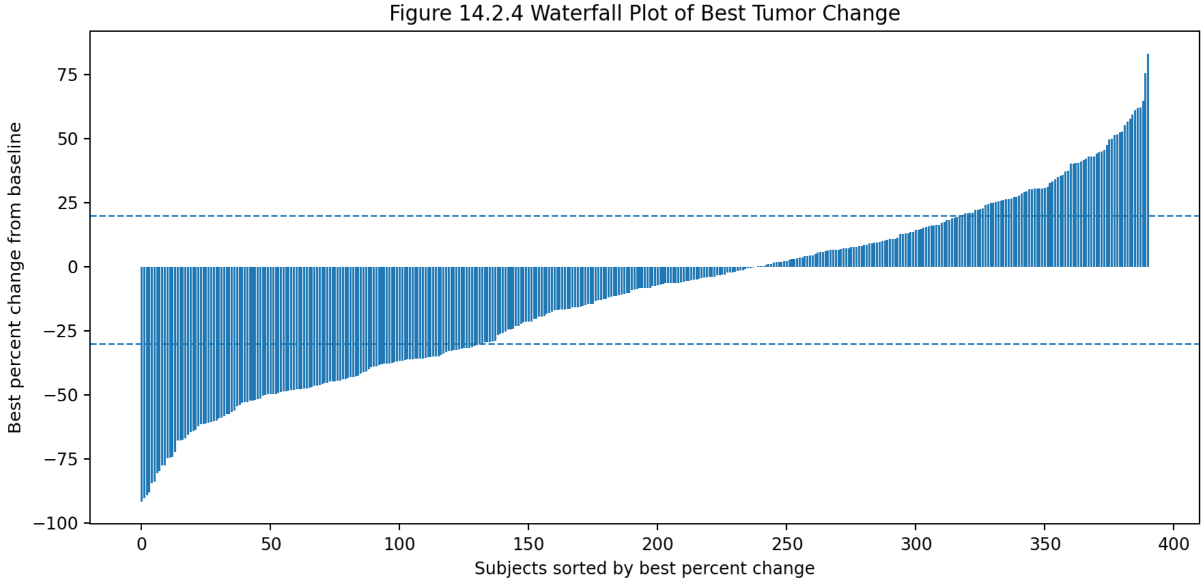
• OS in the simulated dataset favors ONC-305 + SOC, with Cox HR 0.67 (0.51, 0.88) and log-rank p-value 0.004.

• ORR is 97 (46.0%); 95% CI 39.1-52.9% for ONC-305 + SOC and 41 (19.6%); 95% CI 14.5-25.7% for Placebo + SOC in the FAS.

Analysis	N	Events, n (%)	Median months (95% CI)	6-mo KM rate	12-mo KM rate	18-mo KM rate
ONC-305 + SOC	211	91 (43.1%)	22.4 (19.2, 24.0)	94.3%	80.6%	61.1%
Placebo + SOC	209	117 (56.0%)	16.9 (15.7, 19.5)	93.3%	70.3%	47.0%
Treatment comparison						
Cox HR, treatment vs control			0.67 (0.51, 0.88)			
Adjusted Cox HR			0.66 (0.50, 0.87)			
Log-rank test p-value			0.004			

Response category	ONC-305 + SOC (N=211)	Placebo + SOC (N=209)	Total (N=420)
CR	10 (4.7%)	5 (2.4%)	15 (3.6%)
PR	87 (41.2%)	36 (17.2%)	123 (29.3%)
SD	69 (32.7%)	82 (39.2%)	151 (36.0%)
PD	37 (17.5%)	65 (31.1%)	102 (24.3%)
NE	8 (3.8%)	21 (10.0%)	29 (6.9%)
Objective response rate (CR + PR)	97 (46.0%); 95% CI 39.1-52.9%	41 (19.6%); 95% CI 14.5-25.7%	138 (32.9%); 95% CI 28.4-37.6%
Disease control rate (CR + PR + SD)	166 (78.7%); 95% CI 72.5-84.0%	123 (58.9%); 95% CI 51.9-65.6%	289 (68.8%); 95% CI 64.1-73.2%

Figure 14.2.4 Waterfall Plot of Best Tumor Change



6. Safety and exposure

• Safety summaries use the Safety Set and actual treatment received.

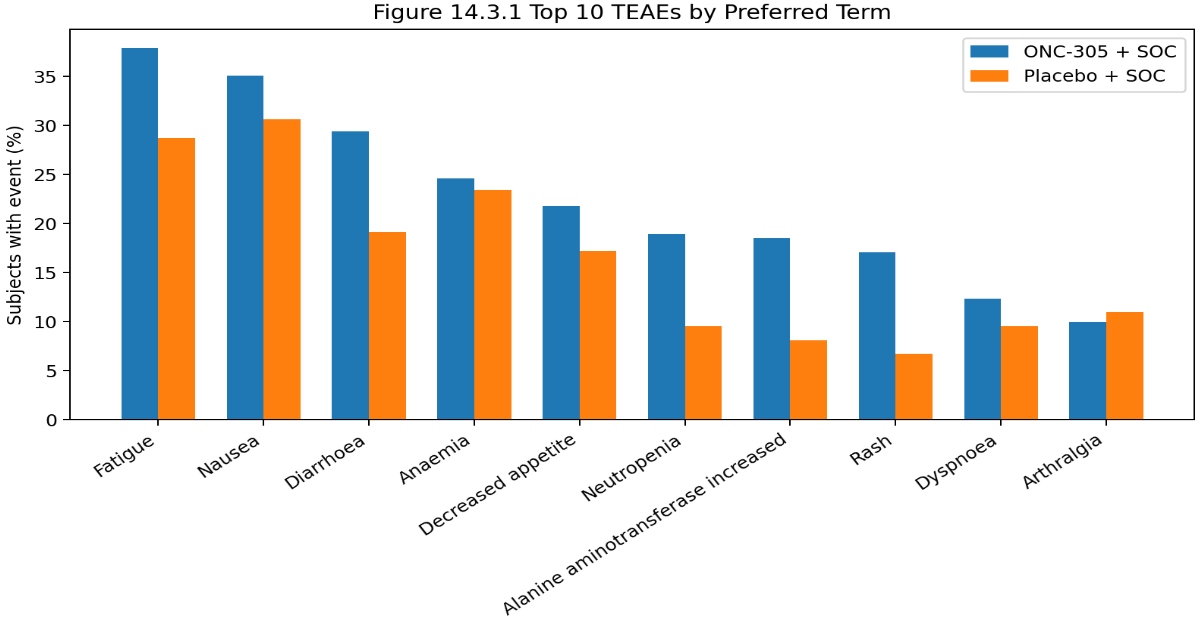
• The active arm has higher treatment-related TEAE incidence in the simulation, as often expected when demonstrating oncology safety reporting.

• Listings are included for deaths, SAEs and AEs leading to discontinuation.

Safety summary	ONC-305 + SOC (N=211)	Placebo + SOC (N=209)	Total (N=420)
Any TEAE	203 (96.2%)	186 (89.0%)	389 (92.6%)
Any treatment-related TEAE	170 (80.6%)	108 (51.7%)	278 (66.2%)
Any grade >=3 TEAE	86 (40.8%)	76 (36.4%)	162 (38.6%)
Any serious TEAE	65 (30.8%)	53 (25.4%)	118 (28.1%)
Any TEAE leading to treatment discontinuation	24 (11.4%)	14 (6.7%)	38 (9.0%)
Any fatal TEAE	6 (2.8%)	5 (2.4%)	11 (2.6%)

Exposure summary	ONC-305 + SOC (N=211)	Placebo + SOC (N=209)	Total (N=420)
Treatment duration, days - mean (SD)	286.9 (179.5)	236.1 (154.6)	261.6 (169.3)
Treatment duration, months - median (Q1, Q3)	8.4 (4.2, 14.0)	6.9 (3.6, 10.6)	7.4 (4.0, 12.5)
Relative dose intensity, % - mean (SD)	90.0 (8.8)	93.5 (8.3)	91.7 (8.7)
Relative dose intensity, % - median (min, max)	90.2 (62.7, 105.0)	94.4 (71.8, 105.0)	91.5 (62.7, 105.0)
RDI <80%	25 (11.8%)	13 (6.2%)	38 (9.0%)
RDI >=90%	107 (50.7%)	137 (65.6%)	244 (58.1%)

Figure 14.3.1 Top 10 TEAEs by Preferred Term



7. Validation, traceability and reviewer readiness

• Validation status: 62 PASS, 0 WARN, 0 FAIL.

• Result-level traceability links outputs to source datasets, analysis populations, methods and programs.

• The package includes an ADaM Reviewer Guide, Programming/QC Plan and TLF Shell Book.

Program validation tracker

Program	Purpose	Role	Validation approach	Status
generate_data.py	Synthetic SDTM-like source data	Primary	Code review + raw_manifest counts	PASS
build_adam.py	ADaM-style datasets and metadata	Primary	Traceability review + variable-level checks	PASS
generate_tlfs.py	Tables/listings/figures	Primary	Output existence + selected hand calculations	PASS
qc_validation.py	Automated validation checks	Primary	PASS/WARN/FAIL tracker	PASS
render_reports.py	Baseline report rendering	Primary	PDF render verification	PASS
render_professional_package.py	Professional SAP/CSR-style package	Primary	PDF render verification + artifact manifest	PASS
programs/sas/*.sas	SAS templates	Template	Provided as regulated-workflow examples; not executed in Python environment	TEMPLATE
programs/r/*.R	R templates	Template	Provided as alternate implementation examples	TEMPLATE

Analysis results metadata excerpt

Result ID	Title	Source	Population	Program
T14.1.1	Demographics and baseline characteristics	ADSL	FAS	generate_tlfs.py
T14.1.2	Subject disposition	ADSL	FAS	generate_tlfs.py
T14.1.3	Extent of exposure	ADSL	Safety	generate_tlfs.py
T14.1.4	Major protocol deviations	Mock deviation log + ADSL	FAS	render_professional_package.py
T14.2.1	Primary analysis of PFS	ADTTE	FAS	generate_tlfs.py
T14.2.2	Overall survival	ADTTE	FAS	generate_tlfs.py
T14.2.3	Best overall response	ADRS	FAS	generate_tlfs.py
F14.2.1	Kaplan-Meier plot for PFS	ADTTE	FAS	generate_tlfs.py
F14.2.3	PFS subgroup forest plot	ADTTE + ADSL	FAS	generate_tlfs.py
T14.3.1	Safety overview	ADAE + ADSL	Safety	generate_tlfs.py
T14.3.2	TEAEs by SOC and PT	ADAE	Safety	generate_tlfs.py
T14.3.4	Laboratory shift summary	ADLB	Safety	generate_tlfs.py
L16.2.1	Deaths listing	ADAE + ADSL	Safety	generate_tlfs.py

8. Discussion, limitations and portfolio conclusion

• The simulated efficacy results demonstrate survival-analysis and response-summary capability. They are not clinical evidence.

• The safety outputs demonstrate subject-level adverse-event reporting and lab-shift logic, not medical conclusions about a real therapy.

• The most job-relevant strength is the full analysis workflow: SAP -> ADaM derivation -> TLFs -> validation -> CSR-style report -> reviewer guide.

• The next optional upgrade would be to export formal XPT datasets and run a standards validation tool locally, then add the validation report to the repository.

Repository appendices

Appendix	Repository location
Full TLF packet	outputs/reports/tlf_packet.pdf
Validation report	outputs/reports/validation_report.pdf
ADaM Reviewer Guide	outputs/reports/adam_reviewer_guide.pdf
Programming/QC Plan	outputs/reports/programming_qc_plan.pdf
TLF Shell Book	outputs/reports/tlf_shell_book.pdf
Traceability metadata	metadata/analysis_results_metadata.csv and source_to_adam_traceability.csv
GitHub guide	docs/github_upload_guide.md